

Family Dynamics at the Bedside

NHP · Health Promotion Across the Lifespan · Family Dynamics

Nurse lens

TEACH
ASSESS
ESCALATE

clinical question

When one person is the client, how does the nurse recognize whether the family is helping or harming that person's health, and what does the nurse do first?

today is not a vocabulary tour

you will use the science only when it explains a patient question or a safety decision.

1

See the whole map first: five lanes

2

Notice what the family is doing to health

3

Decide what the nurse does first

Opening case: an 8-year-old with abdominal pain

patient says

An 8-year-old is admitted with abdominal pain. The mother is present. During the history the nurse learns the parents are divorcing.

your first task

What do you still need to know before you can plan care?

what you know so far

- Abdominal pain: onset, location, duration, character, what makes it better or worse
- One parent at the bedside
- Parents divorcing

what you do not know yet

- Living situation and household
- How family members interact
- What the child believes about the divorce
- Strengths, stressors, supports

Before facts: decide what information is missing and what could be unsafe.

The organizing question

When one person is the client, how does the nurse recognize whether the family is helping or harming that person's health, and what does the nurse do first?

Structure

who counts as family; roles; five functions

Dynamics

healthy vs unhealthy patterns; resources

Risk

ACEs; risk and protective factors; parenting

Strain

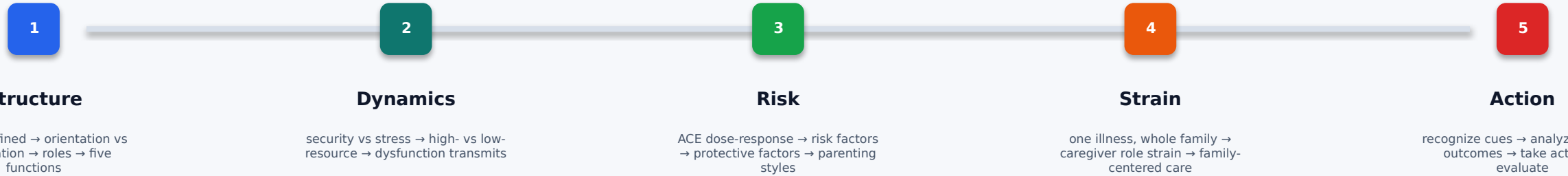
illness hits the whole family; caregiver role strain

Action

assess; diagnose; outcomes; intervene; evaluate

Every concept must answer: what do I teach, what do I assess, what must I escalate?

Chapter map: structure to action



Map first. Drill down only where the map or performance data shows a weak link.

Structure: who is family, and what does a family do?

Economic support

Meeting members' material needs.

Emotional support

Intimacy: mutually shared trust that buffers outside stress.

Socialization

First teacher of norms, values, and how emotions are shown. Can also pass on unhealthy behavior.

Sexuality and reproduction

Family norms around partnership and reproduction.

Ascribed social status

Status at birth versus achieved status by effort; family support shapes achievement.

Identify family structure, roles, and the five family functions.

Structure: roles, culture, and the nurse's own bias

Informal roles

Decision-maker, peace-maker, tradition-holder.

Dysfunction roles

Hero, mascot, scapegoat, lost child, enabler, parentified child.

Culture

Shared values, norms, symbols, language, and way of life passed between generations.

The nurse's lens

Cultural humility plus inclusiveness; watch for implicit bias.

Recognize informal and unhealthy family roles and the nurse's obligation to cultural humility.

Dynamics: healthy or unhealthy?

healthy: security and comfort

- mutual respect
- trust
- caring communication
- needs met, or help sought
- resiliency plus resources

unhealthy: stress

- arguments, intrusion, criticism
- unfriendly, disrespectful, hostile interaction
- needs not effectively met
- caregiver trauma reaching the child

One-minute prompt: explain this distinction to a patient without using the word 'dangerous' loosely.

Risk: adverse childhood experiences are dose-dependent

caregiver history

abused as a child; young caregivers; limited understanding of child development

household strain

single parent, low income or education, financial stress, special-needs caregiving

discipline and supervision

inconsistent discipline or spanking, low supervision

**how many ACEs, not
which one**

family climate

isolation, high conflict, attitudes that accept violence

community

violence, poverty, food insecurity, drug and alcohol access, unstable housing

opportunity

few youth activities, low involvement, limited opportunity

Risk is a pattern: exposure, timing, dose, susceptibility, and clinical context.

Risk: protective factors the nurse can build

in the family

needs met; stable nurturing relationships; social support; shared activities; school valued; peaceful conflict resolution; mentors

in the community

parenting education; financial help; health and mental health services; safe housing and childcare; preschool and after-school programs

when parents divorce

no sides, no arguments in front of the child; talk early and often; not the child's fault; keep routine and promises

safety teaching limit

Anticipatory guidance is a nursing action. Give it in plain terms and check what the parent heard.

Write two sentences of anticipatory guidance for the divorcing parents in the opening case.

Risk: four parenting styles on two axes

Uninvolved

Low warmth, low control. Poor outcomes in all domains.

Permissive

High warmth, low control. Behavior problems, lower academics; higher self-esteem and social skills.

Authoritarian

Low warmth, high control. Moderate academics; poorer social skills, lower self-esteem, more depression.

Authoritative

High warmth, high control, explains rationale. Best outcomes; lowest underage drinking risk.

Compare parenting styles by responsiveness and demandingness and their outcomes.

Risk: what positive discipline looks like

1. set the guideline

clear rule plus a warning

2. name the consequence

specific if-then that fits the behavior

3. follow through

immediately, calmly

4. reinforce

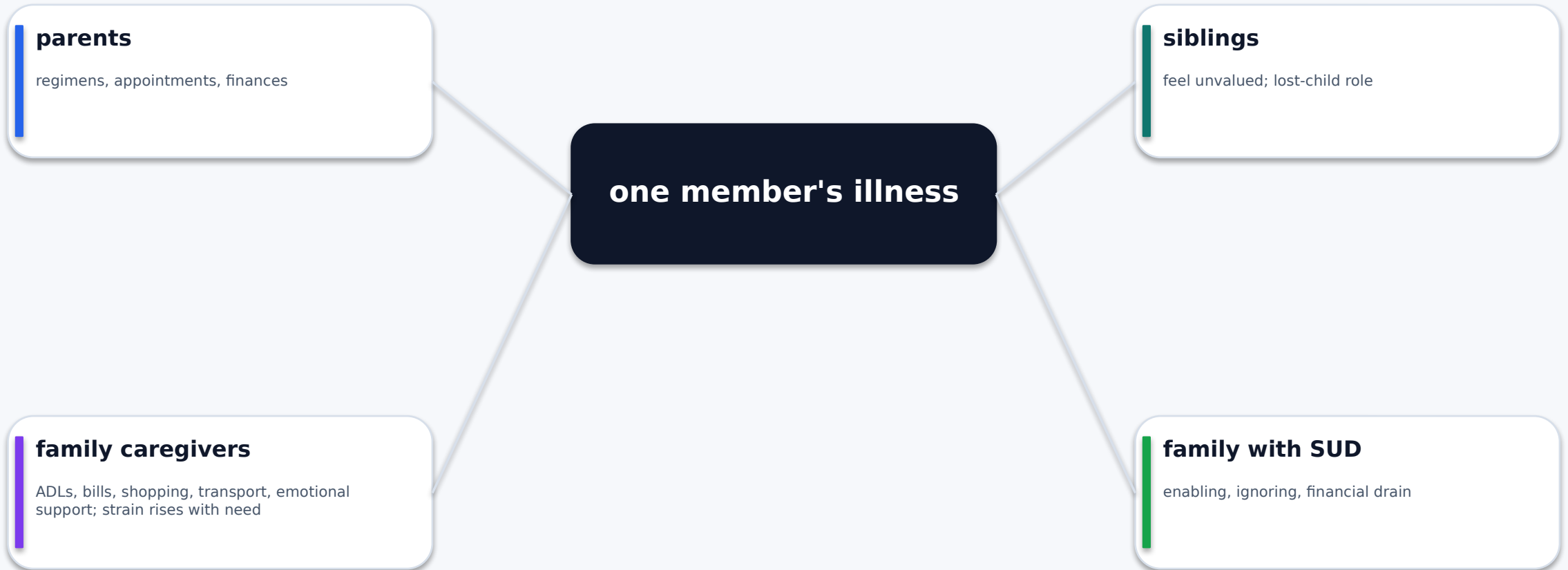
positive attention for the behavior you want

5. teach the feeling

after a tantrum, name the emotion and show an acceptable way to express it

Accurate + plain + nonjudgmental beats technical + vague.

Strain: one member's illness reaches the whole family



Control systems explain timing, symptoms, and safe teaching.

Strain: caregiver role strain and what the nurse offers

Stress signs

anger, withdrawal, anxiety, depression, exhaustion, sleeplessness, irritability, new illness

Resources

day care, respite, residential or palliative care; Al-Anon, Nar-Anon, Sibshop; family therapy

Self-care teaching

provider visits, nutrition, exercise, rest; relaxation breathing, visualization, meditation

Family-centered care

respect and dignity; collaboration; empowerment; information sharing

Recognize caregiver stress signs and match them to resources and self-care teaching.

Action: the nursing process is the clinical judgment model



Map first. Drill down only where the map or performance data shows a weak link.

Action: assess the family without losing the client

mini case

The 8-year-old from the opening case. Mother present. Parents divorcing. The nurse is about to take the family history.

cues

- Minor: parent decides; autonomy still advocated
- Survey hygiene, affect, communication, nutrition
- What does the family know, and how do they respond?
- Supports: respite, community, other family

your task

Put the steps in order and say which one protects the client.

consider

- Ask the client privately who they want present
- General survey of client and family
- Hostile or disrespectful interaction is a cue to act on

Which single step must happen before any family member is interviewed?

Action: sort the cues by what the nurse does next

cues to sort

- asks whether spanking is acceptable
- bruises in different stages of healing, inconsistent explanation
- sleeplessness, irritability, dread about the future
- hostility and contempt toward the child during the survey

response ladder: most to least urgent

- mandated report per state law and agency policy
- notify provider; social work or case management referral
- caregiver role strain: resources and self-care teaching
- teaching moment: positive discipline

Order the four cues from most to least urgent nursing action.

Checkpoint: the caregiver in the waiting room

checkpoint question

An adult daughter caring for her father with dementia says she has not slept properly in weeks, snaps at him, and 'cannot see how this ends.' What is the nurse's best response?

- A** A. Reassure her that all caregivers feel this way and it will pass
- B** B. Suggest she try harder to keep a routine at home
- C** C. Identify caregiver role strain and offer respite care and support-group information
- D** D. Tell her that her father would be safer in a residential facility

Select one.

Capstone: the abdominal pain case, resolved

capstone scenario

During the family history the mother says, in front of the child, that the pain started when his father left and that it is his fault for upsetting everyone.

priority nursing response

What is the nurse's priority action?

- A** A. Correct the mother immediately in front of the child so the child hears it is not their fault
- B** B. Speak with the child privately, then address the parent's statement and involve social work per policy
- C** C. Document the statement and continue the physical assessment
- D** D. File a mandated report for emotional abuse before doing anything else

Select one.

Rapid retrieval: five-item exit check

Name the five family functions

State the one test that separates healthy from unhealthy dynamics

Why does the number of ACEs matter more than which one?

Name three caregiver stress signs

What must the nurse do before interviewing any family member?

Answer all five.

Takeaway: what the nurse notices, and what the nurse does first

Notice

roles, functions, interaction patterns, caregiver strain

Sort

mandated report, provider and social work referral, resources, teaching

Protect

privacy first; ask the client who they want present

Teach

protective factors, positive discipline, caregiver self-care

final bedside frame

A safe nurse assesses the family as part of the client, sorts cues by required action, and teaches what protects.